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Correct
Answer

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Practice Exam - 1

Question 108 of 117



A 45-year-old gentleman with a history of non-ischemic cardiomyopathy presents to the office after recent admission for treatment of acute decompensated heart failure. This was his 3rd hospitalization for HF in the past year. He was previously on guideline directed medical therapy with carvedilol, sacubitril-valsartan, and spironolactone but these were discontinued during the admission due to hypotension and he was discharged on low-dose metoprolol succinate. Despite increasing doses of bumetanide, the patient continues to complain of dyspnea with minimal exertion, orthopnea, PND, and weight gain. Exam is notable for BP 92/78, HR 89, elevated JVP, and warm extremities. ECG demonstrates atrial fibrillation, QRS 94 ms, LVH, and abnormal R wave progression. TTE performed during recent hospitalization revealed LVEDD 6.6 cm, LVEF 23%, normal RV size and function, and severe mitral regurgitation.

Which of the following treatments is indicated for this patient:

A. Septal myectomy and ventricular reconstruction



B. CRT-D

C. Transcatheter mitral valve repair

D. Intermittent dobutamine infusion q2 weeks



E. Cardiac transplantation

Commentary

References

TESTING POINT: Answer: E. Cardiac Transplantation The goal of the question is to recognize a patient with NYHA Class III HF who demonstrates signs and symptoms of advanced disease and refer them for appropriate advanced therapies. Even in the presence of mitral regurgitation or other secondary phenomenon, it may not be appropriate to refer for procedures if a patient's underlying cardiomyopathy is advanced. Answer A: There is no role for septal myomyectomy in non-ischemic cardiomyopathy; this is a procedure referred for patients with hypertrophic cardiomyopathy and outflow obstruction due to asymmetric septal hypertrophy. Ventricular reconstruction is a procedure previously tested to reduce the volume of a dilated ventricle. It has not been shown to alter long term morbidity or mortality. Answer B: The patient has a QRS duration of 94 ms, and no indication for CRT. He should have a defibrillator implanted if he does not currently have one. Answer C: Transcatheter mitral valve repair is not indicated in advanced heart failure patients not tolerating GDMT. These patients were not enrolled in the COAPT trial. Answer D: There is no role for intermittent inotrope infusions in advanced heart failure. If the patient is not a candidate and wishes to pursue palliative therapy with inotropes, they should be administered continuously, not intermittently.

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